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Epidural Steroid Injections: Patient Information Leaflet

This leaflet provides essential information about three different types of Epidural Steroid Injections (ESI) for pain management. Your pain specialist will discuss a specific treatment plan with you.

What Are Epidural Steroid Injections and why are these done?

Epidural steroid injections (ESIs) deliver cortisone and a small amount of local anesthetic into the 'epidural space' (around the spinal cord and nerves) to reduce swelling and pain.

Three types include:

1. **Transforaminal ESI:** Targets specific nerve roots as they exit the spine – *These injections are done to treat sciatica in the legs or nerve pain in the arms due to herniated discs, these injections are target specific.*
2. **Interlaminar ESI:** Injected into the neck (cervical), upper back (thoracic) or lower back (lumbar) – *Can potentially help in reducing pain due to spinal stenosis caused by bulging discs and degenerative disc disease. Sometimes pain specialist use these as an alternative to foraminal ESI (as above).*
3. **Caudal ESI:** Administered through the tailbone (sacral hiatus) in the epidural space – *this is a different approach to epidural space and helps in similar ways as Interlaminar epidurals (as above) do.*

These aim to reduce inflammation, improve mobility, and delay/avoid surgery. Often paired with physical therapy or medications.

Success Rates

1. 50–80% of patients experience pain relief, meaning some patients might NOT see any benefit from these injections.
2. It takes 4-6 weeks for these injections to work.
3. Relief lasts from a few weeks to months, repeat injections may be needed.
4. Some patients are cured and would not need repeat injections.

Note: Results vary based on condition severity and overall health. Surgical referral might be needed if no or very short-lived pain relief is achieved.

Pre-Procedure Preparation

1. Blood thinners might need to be stopped please inform your doctor if you are on any. We will provide instructions to stop if needed.
2. If you are too anxious about the procedure, please let your doctor know in advance to get a pill under the tongue for relaxation 30 minutes before the procedure. Please do not drive once you had the pill.
3. Arrange a ride home. You should not drive for 24 hrs after the procedure.
4. Inform Clinic: If pregnant, allergic to contrast, steroids or Chlorhexidine or if taking diabetes medications.

How the Procedure Works

1. No sedation: You'll be awake but given local anesthetic, unless you had a mild relaxant under the tongue. Most injections are not painful.
2. You would lie on your stomach on an X-ray (fluoroscopy) table.
3. Your skin will be cleansed and then we will inject local anesthetic to numb the area.
4. Guided by fluoroscopy, a needle is placed into the epidural space. You might feel slight nerve pain or micro shock if you are having transforaminal ESI if the needle touches the nerve.
5. Contrast dye confirms correct placement.
6. Steroid medication is injected.

Procedure Duration: 15–30 minutes. Mild pressure/discomfort may occur.

Procedural Difficulties

1. At times it is not possible to perform the injection as planned.
2. At times we perform a plan B injection.

Your doctor will speak to you if a situation like that arose.

Post-Procedure Precautions

1. Keep the dressing on for 24 hrs.
2. Rest: Avoid strenuous activity for 24 hours.
3. Driving: Do not drive for 24 hours.
4. Pain Relief: Ice the area if sore. Pain may temporarily flare before improving. Keep taking your pain medications.
5. Resume normal activities gradually; attend physical therapy if prescribed.

Follow-up

The pain clinic will schedule a follow-up with you in 4-6 weeks.

Risks and Side Effects

Common (temporary):

1. Mild pain/swelling at injection site
2. Temporary Flare up of pain – if that happens, we will prescribe extra pain relief if needed.
3. Flushing, insomnia, or temporary blood sugar spikes (diabetics: monitor closely).

Side effects of repeated cortisone injections

1. Increased blood sugars if you are a diabetic
2. Osteoporosis – weak bones
3. Suppression of your immune system (delayed healing)
4. Fluid retention and weight gain
5. Cataracts and skin changes

Rare (seek help if experienced):

1. Infection, bleeding with a blood clot (epidural hematoma), or nerve damage.
2. Severe headache (worse when upright and relieved when you lie flat), allergic reaction (rash, breathing difficulty).
3. Flare-up with worsening pain
4. Any new onset weakness.

When to contact the Pain Clinic or the ER

Contact the Pain Clinic if:

1. Fever >100.4°F, redness/swelling at injection site
2. Severe rash or suspected allergic reaction
3. Worsening pain after 3–5 days.

Go to the Emergency Room if:

1. Severe headache, chest pain, or trouble breathing.
2. Sudden leg weakness/numbness or loss of bladder/bowel control or severe back or leg pain

Questions? Contact Your Care Team

Before/after procedure:

 782-377-7813

 Info@redsandspm.com

In an emergency

→ Call 911, or visit your local emergency department